

PRIMARY TREATING PHYSICIAN'S PROGRESS REPORT (PR-2)

Check the boxes which indicate why you are submitting a report at this time. If the patient is "Permanent and Stationary" (i.e., has reached maximum medical improvement), do not use this form. You may use DWC Forms PR-3 or PR-4.

- ☐ Periodic Report (Required 45 days after last report)
 ☐ Change in treatment plan
 ☐ Release From Care
☐ Change in work status
 ☐ Need for referral or consultation
 ☐ Response to request for information
☐ Change in patient's condition
 ☐ Need for surgery or hospitalization
 ☐ Request for authorization
☐ Other _____

Patient

Testcase

QA-BOT-Maria

Patient last name:

Patient first name:

MI

Patient Street Address/PO Box

Patient City

State

Zip Code

Sex

Occupation

(949) 555-0134

Phone Number

Date of Birth

04/12/1985

Claims Administrator

Date of Injury

08/15/2026

QA-BOT Test Claims Admin

QA-BOT-0001

Claims Administrator Name

Claim number

Claims Administrator Street Address/

Claims Administrator City

State

Zip Code

Phone Number

Fax Number

Employee Name

Phone Number

Subjective Complaints (The information below must be provided. You may use this form or you may substitute or append a narrative report):

At the initial encounter dated 2026-09-07, the injured worker reported lower back pain after lifting a heavy box at work, worse with flexion, radiating to the left knee. Subjective complaints for the most recent encounter dated 2026-09-13 were not documented.

Objective findings: (Include significant physical examination, laboratory, imaging, or other diagnostic findings.)

Objective findings from the most recent encounter dated 2026-09-13 were not documented. The prior documented examination from 2026-09-07 noted lumbar paraspinal tenderness, reduced flexion, positive left straight-leg raise, and mild left knee effusion.

Diagnoses:

- | | | | | | | | |
|----|----------------------|--------|------------------------------------|-----|----------------------|--------|----------------------|
| 1. | <input type="text"/> | ICD-10 | <input type="text" value="M54.5"/> | 7. | <input type="text"/> | ICD-10 | <input type="text"/> |
| 2. | <input type="text"/> | ICD-10 | <input type="text"/> | 8. | <input type="text"/> | ICD-10 | <input type="text"/> |
| 3. | <input type="text"/> | ICD-10 | <input type="text"/> | 9. | <input type="text"/> | ICD-10 | <input type="text"/> |
| 4. | <input type="text"/> | ICD-10 | <input type="text"/> | 10. | <input type="text"/> | ICD-10 | <input type="text"/> |
| 5. | <input type="text"/> | ICD-10 | <input type="text"/> | 11. | <input type="text"/> | ICD-10 | <input type="text"/> |
| 6. | <input type="text"/> | ICD-10 | <input type="text"/> | 12. | <input type="text"/> | ICD-10 | <input type="text"/> |

Treatment Plan: Include treatment rendered to date. List methods, frequency and duration of planned treatment(s). Specify consultation/referral, surgery, and hospitalization. Identify each physician and non-physician provider. Specify type, frequency and duration of physical medicine services (e.g., physical therapy, manipulation, acupuncture). Use of CPT codes is encouraged. Have there been any **changes** in treatment plan? If so, why?

Treatment rendered since the last report is not documented in the 2026-09-13 follow-up encounter. The active treatment plan remains in active status with goals to restore lumbar range of motion and reduce pain and to progress to an independent home exercise program over 6 weeks. Planned treatment includes physical therapy for lumbar stabilization, CPT 97110, at a frequency of 2 times per week for 6 weeks, quantity 12. The initial encounter treatment plan text recommends conservative care focused on active recovery, including NSAID therapy as first-line treatment for a short course of 7 to 14 days if there are no contraindications, consideration of a short nocturnal trial of a non-carisoprodol muscle relaxant for up to 2 weeks if pain remains moderate to severe despite NSAIDs, and monitoring of the left knee with separate evaluation if symptoms persist or worsen. The active treatment plan notes that MTUS Low Back chapter supports a time-limited course of active physical therapy for acute lumbar strain.

Work Status: This patient has been instructed to:

- ☐ Remain off-work until _____
- ☒ Return to *modified* work on _____ with the following limitations or restrictions. (List all specific restrictions re: standing, sitting, bending, use of hands, etc.):

No lifting > 10 lbs; no repetitive bending or twisting; sit/stand as tolerated.

- ☐ Return to full duty on _____ with no limitations or restrictions.

Primary Treating Physician:: (original signature, do not stamp)

Date of Exam 09/13/2026

I declare under penalty of perjury that this report is true and correct to the best of my knowledge and that I have not violated Labor Code section 1523.

Physician signature _____

Cal. License Number: _____

Executed at: _____

Date (mm/dd/yyyy): _____

Physician name _____

Specialty: _____

Physician address: _____

Phone Number _____

PRIVACY NOTICE: A statement of current data collection and use policies and certain privacy rights of injured workers may be found at the following website: http://www.dir.ca.gov/od_pub/privacy.html.

These pages are an attachment to DWC Form PR-2 and are not part of it. Form PR-2 is the State of California's document; everything here is ChironAI's supporting analysis, prepared under the direction of the treating physician. No field of the state's form is reproduced or restated on these pages. Read them with the form, never instead of it.

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■ History of Injury

The documented injury date is 2026-08-15. The mechanism of injury is documented as lifting a 40 lb box and feeling a pop in the lower back. The most recent follow-up encounter dated 2026-09-13 describes a single-event injury with immediate onset, specifically: lifting a 40 lb box at work with a sudden pop in the lower back. Job duties documented in the encounter context include repetitive lifting up to 50 lb, frequent bending, and prolonged standing.

■ Causation Analysis

A physician-reviewed and approved causation analysis finalized on 2026-09-07 states that the documented lumbar complaint is industrial. The final rationale states that the work activity involved lifting a 40 lb box with an immediate pop in the lower back, and that the mechanism and timing support work-related causation for the lumbar condition. The final industrial percentage documented in that analysis is 75%. The same analysis states that although the left knee is listed as an affected body part, no knee-specific mechanism, symptoms, or diagnosis were provided, so no reliable industrial causation opinion can be made for the knee from the available data.

■ Physician Opinion

This PR-2 follow-up record dated 2026-09-13 contains limited clinical documentation. The current condition at that visit is not described with updated subjective complaints, examination findings, treatment rendered, or response to treatment. Based on the available records, the documented industrial condition is the lumbar injury associated with the lifting event, with ongoing conservative management through an active physical therapy plan and modified work restrictions. Continued treatment appears to be planned in the form of lumbar stabilization physical therapy 2 times per week for 6 weeks, with work restrictions of no lifting greater than 10 pounds, no repetitive bending or twisting, and sit/stand as tolerated. Left knee involvement remains insufficiently documented for a definitive diagnosis or causation opinion in the available records.

■ Physician Attestation

Physician Signature _____

Date _____

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DRAFT — PENDING PHYSICIAN SIGNATURE